

Infection Prevention and Control Policy

Respiratory Viruses (Non-Endemic)

(avian influenza, Nipah and other emerging respiratory viral infections causing severe respiratory disease)

Scope

Site	Service/Department/Unit	Disciplines
SCGH	Hospital-Wide	All Staff

Contents

Introduction.....	2
Policy Principles	2
1. Patient Assessment.....	2
2. Patient Management	3
3. Airborne Precautions	3
4. Hand Hygiene	4
5. Personal Protective Equipment (PPE).....	4
6. Donning and removing PPE.....	5
7. Aerosol generating procedures.....	5
8. Cough Etiquette.....	6
9. Disposal of waste	6
10. Linen	6
11. Catering	6
12. Equipment	6
13. Specimen collection	6
14. Care of the deceased	7
15. Transfer of patients to other departments	7
16. Visitors (Ward areas)	7
17. Environmental cleaning	7
18. Staff Health.....	7
Related Documents	8
References.....	8
Authorisation	9
APPENDIX 1: Collection of Specimens	10
APPENDIX 2: Staff Register.....	12

Introduction

Certain rare or novel viruses that are transmitted via respiratory secretions have the ability to cause severe disease in humans. Mostly these viruses are not endemic in Australia but occur overseas. Recent examples include; Nipah virus, Middle East Respiratory Syndrome coronavirus (MERS-CoV), avian influenza (influenza A/H5N1 and A/H7N9) and Severe Acute Respiratory Syndrome (SARS) coronavirus. These viruses often derive from animals (zoonoses) and cause severe disease in humans, usually pneumonia, but sometimes involve other organ systems.

To date, the respiratory viruses have emerged overseas, so are most likely to be seen in Australia in people following recent overseas travel. News of the discovery of a novel respiratory virus is likely to be rapidly transmitted around the world allowing the Department of Health to distribute warnings and information, such as geographic risk areas, prior to their arrival in WA. However, it is important to remain vigilant as there is always the possibility that a traveller infected with a novel virus will arrive in WA without warning.^{1,2, 3}

Policy Principles

This policy provides details on the infection prevention and management of non-endemic viruses that may cause severe respiratory disease.

This policy relates to all staff involved in the management of these respiratory viruses at SCGH.

1. Patient Assessment

These respiratory viral infections most commonly present as acute respiratory distress syndrome within days or several weeks of arriving from overseas. A risk assessment of recent overseas travellers (usually within the last 14 days) with an acute respiratory illness who present to the Emergency Department (ED) should be performed immediately.

For a known outbreak of a severe respiratory disease virus overseas, the Department of Health may develop guidelines for Emergency Departments including case definitions, which are usually based on symptoms, incubation period and travel history. Additional assessment and management guidelines may be provided by the Infection Prevention and Management (IPM) unit where guidelines are unavailable, advice can be sought from the IPM unit, on-call clinical microbiologist or infectious diseases physician.

Following the risk assessment, the IPM unit or on-call clinical microbiologist **must be notified immediately** of any suspected case of non-endemic respiratory virus infection. They will help coordinate testing, the infection control response, and liaison with the Public and Aboriginal Health Division of WA Health. Early consultation with the Infectious Diseases service should be considered.

The differential diagnosis of acute respiratory illness in a traveller includes common viral aetiologies such as seasonal influenza that also occur locally. If the risk assessment determines that the patient does not have a suspected non-endemic respiratory virus infection, they should be managed in accordance with the SCGH IP&M Influenza-like Illness Policy.

2. Patient Management

Patient admission

Patients presenting with a probable or suspected non-endemic respiratory virus infection should be identified immediately and given a surgical face mask to wear. Triage the patient to the Airborne Infection Isolation Room (AIIR) in the ED for immediate assessment.

Ensure the door of the AIIR is closed and the isolation mode (negative pressure) is activated and functioning correctly. Place a blue isolation precaution card (Airborne Precautions) outside the patient room – explain the reasons for isolation to the patient and accompanying persons. Accompanying persons should also be given a surgical facemask to wear.

Healthcare workers (HCWs) attending patients with a suspected or confirmed non-endemic respiratory virus infection must wear a long sleeve fluid repellent gown, P2 (N95) respirator mask (fit checked), disposable full face visor and gloves (ensure the gloves cover the cuffs of the gown sleeve). Refer to Section 5. All PPE must be put on and removed in the ante-room of the AIIR.

Transporting patients

The patient should wear a surgical face mask during transportation if possible. Patients on oxygen therapy should be changed to nasal prongs, if safe to do so, and wear a surgical face mask. **Accompanying staff** must wear a long sleeve fluid repellent gown, P2 (N95) respirator mask (fit checked), disposable full face visor and gloves (ensure the gloves cover the cuffs of the gown sleeve). Refer to Section 5. A new set of clean PPE must be donned prior to transporting the patient.

Transfer the patient to the ward via the Gold lifts. **Ensure no visitors or other staff are in the lift during transfer.**

Direct admissions to SCGH

Patients with a suspected or confirmed case of non-endemic respiratory virus infection that have been assessed outside SCGH will bypass ED and be admitted directly to the AIIR on either G54 or ICU, as clinically determined. Another AIIR is located on G71.

NOTE: If all AIIRs are in use contact the Infection Prevention and Management Unit or the on-call clinical microbiologist for advice.

Persons accompanying patient

In general no visitors would be allowed into the AIIR. If this is unavoidable for medical reasons the accompanying person(s) must be trained to wear personal protective equipment (PPE) as per Section 5.

3. Airborne Precautions

Place a blue 'airborne precautions' sign on the door outside the AIIR. The door must be kept closed and negative pressure activated.

Non-essential staff must not enter the AIIR e.g. students and agency staff (refer to full list of staff restrictions in Section 18).

Duration of Airborne Precautions

Airborne precautions should only be discontinued on advice from the Infection Prevention and Control Unit or a clinical microbiologist.

4. Hand Hygiene

Perform hand hygiene in accordance with the '5 moments for hand hygiene'. Use handwash solution or alcohol based hand rub.

5. Personal Protective Equipment (PPE)

Staff entering the AIIR must wear PPE including a P2 (N95) respirator mask (fit checked), disposable full face visor, a long sleeve fluid repellent gown and gloves.

Staff must not touch their eyes, nose, mouth or hair while wearing PPE. Staff with beards can wear a powered air purifying respirator (PAPR).

Note: A new set of clean PPE must be donned prior to transporting the patient.

All PPE must be donned and removed in the anteroom to the AIIR. Hand hygiene must be performed before donning and doffing PPE.

P2 (N95) respirator mask

- A close fitting P2 (N95) respirator mask must be worn.
- A fit check must be performed prior to entering the AIIR and every time a new mask is worn. Refer to the instructions on the poster displayed in the anteroom of the AIIR.

Do not re-adjust mask once in the AIIR. Staff who are unable to achieve the correct fit (e.g. due to facial hair or other problems with fitting the respirator) must not enter the AIIR.

If the respirator is damaged, soiled or damp leave the room and change the respirator;

- remove gloves
- apply alcohol based hand rub to hands,
- remove the respirator by the straps
- apply alcohol based hand rub to hands
- put on the new respirator
- put on clean gloves.

Powered air purifying respirator (PAPR)

These may be required for some high-risk pathogens but are not routinely used unless the HCW is unable to satisfactorily achieve a fit check with a P2 (N95) respirator mask (refer to PAPR guidelines for use in SCGH Infection Prevention and Control Manual).

Protective eyewear

A disposable full face visor must be worn by all HCWs.

Long sleeve fluid repellent gowns

- Long sleeve fluid repellent cuffed gowns must be worn by all HCWs.
- Gowns are to be worn once and then placed in clinical waste.

Gloves

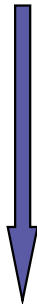
- Gloves must be worn by all HCWs.
- Hand hygiene must be performed after removal of gloves.

- Ensure gloves cover the cuffs of the gown sleeves.

6. Donning and removing PPE

All PPE needs to be donned and removed in the AIIR anteroom. Refer to Table 1 for the sequence of donning and removing PPE.

Table 1: Sequence for donning and removing PPE

Donning PPE		Removing PPE	
	Hand hygiene		Gloves – turn inside out by cuff; discard into clinical waste
	Long sleeve fluid repellent gown		Hand hygiene
	P2 (N95) respirator mask perform fit check		Gown- turning inside itself, discard into clinical waste
	Protective eyewear (full face visor) Recheck respirator		Hand hygiene
	Gloves		Full face visor (disposable), discard into clinical waste
			Hand hygiene
			P2 (N95) respirator mask handle straps/ties only, discard into clinical waste
			Hand hygiene

7. Aerosol generating procedures

Aerosol generating procedures (AGPs) are those that stimulate coughing and promote the generation of fine airborne particles (aerosols) resulting in the risk of airborne transmission. Procedures include nebulised medication, endotracheal intubation, mechanical ventilation, airway suctioning or opening a ventilator circuit, bronchoscopy, diagnostic sputum induction, positive pressure ventilation via face mask (non-invasive ventilation), and high frequency oscillatory ventilation.

AGPs must not be undertaken on suspected or confirmed severe respiratory virus disease infection cases unless strictly necessary and authorised by the Medical Consultant. Any procedures undertaken on cases that may produce aerosols (e.g. nebulisation, suctioning) must be performed with full respiratory precautions in the AIIR (staff to wear P2 (N95) respirator mask, full face visor, gloves, and a long sleeve fluid repellent gown).

Note: Administration of medication via nebulisers is not recommended, use spacers where possible.

8. Cough Etiquette

Encourage the patient to cover their nose and mouth with a tissue when coughing. Promptly dispose of used tissues into the waste bin. Perform hand hygiene after contact with respiratory secretions.

9. Disposal of waste

Place all waste into the clinical waste bin.

10. Linen

Dedicate a linen skip to the AIIR. All linen must be placed in a plastic bag and then into the linen bag. Ensure the linen bag is tied securely. Return to the laundry in the usual way.

11. Catering

- Catering staff must not enter the AIIR. Nursing staff are to deliver and collect the patient's meal tray, crockery and cutlery and **must wear PPE** as per Section 5.
- Disposable crockery and cutlery are not required.
- Crockery and cutlery may be returned to the kitchen in the usual way. The combination of hot water and detergents used in the automatic dishwashers is sufficient to decontaminate these items.

12. Equipment

- Keep equipment in the AIIR to a minimum
- Supplies and equipment are to be disposable where possible
- Dedicate non-critical equipment to the room e.g. stethoscope, tourniquet
- Patient charts must be left outside the AIIR and the anteroom
- Reusable equipment, e.g. pulse oximeter is to be cleaned with surface cleaner or detergent wipes followed by disinfection with alcohol impregnated wipes and allowed to dry before use on another patient
- Unused disposable items e.g. dressings, kidney dishes to be discarded on patient's discharge
- If any equipment is to be returned to Sterilisation Services place in plastic bag and then return in the usual way. If equipment cannot be returned to Sterilisation Services contact the Infection Prevention and Control Nurses for advice.

13. Specimen collection

The on-call clinical microbiologist **MUST** be informed of any suspected case of non-endemic respiratory virus infection before specimens are sent to the laboratory for testing. During a known outbreak, specific specimen collection guidelines may be provided. In the absence of specific guidelines, refer to Appendix 1 for generic collection guidelines or contact the on-call clinical microbiologist if still in doubt. If not done already, testing for common respiratory viruses (e.g. influenza, respiratory syncytial virus) should also be performed on the specimens.

During specimen collection, staff must wear PPE as per Section 5. Specimens should be transported directly to Central Specimen Reception Area (CSRA) by a HSA. **DO NOT** use the pneumatic tube system.

14. Care of the deceased

Staff must wear PPE as per Section 5. In the event of death a surgical face mask is to be placed on the deceased prior to movement of the body and the patient placed in a body bag, this must be completely sealed. Inform the mortuary prior to transfer of patients if the patient is suspected or confirmed to have a severe respiratory virus disease.

15. Transfer of patients to other departments

- this should be kept to a minimum
- the receiving department must be informed prior to transfer
- the patient should wear a surgical face mask during transportation if possible. Patients on oxygen therapy should be changed to nasal prongs, if safe to do so, and wear a surgical face mask.
- **Accompanying staff** must wear a long sleeve fluid repellent gown, P2 (N95) respirator mask (fit checked), disposable full face visor and gloves (ensure the gloves cover the cuffs of the gown sleeve). Refer to Section 5.
- A new set of clean PPE must be donned prior to transporting the patient.
- **Ensure no visitors or other staff are in the lift during transfer.**
- if the patient is being transferred to a different health care facility the receiving facility must be notified prior to transport.

16. Visitors (Ward areas)

Refer to Section 2 Patient Management.

NOTE: close contacts of probable and confirmed cases may be subject to follow up by the Public Health Unit. Refer to the latest information for the specific disease.

17. Environmental cleaning

- On discharge, leave room vacant for 30 minutes prior to disinfection cleaning. Leave Isolation mode ON.
- Cleaning staff must wear PPE as per Section 5.
- Follow Standard Operating Procedure cleaning guidelines for isolation room.
- All disposable items in the room are to be discarded on discharge.
- A hydrogen peroxide disinfection clean (Nocospray®) **is not required** on discharge.

18. Staff Health

General

It is strongly recommended that all HCWs receive annual vaccination for seasonal influenza. Only essential medical and nursing staff should enter the patient's AIIR and anteroom.

The following staff should not care for suspected or confirmed cases;

- pregnant women
- individuals with chronic respiratory conditions including asthma, chronic obstructive pulmonary disease (COPD)
- persons with morbid obesity

- agency staff or students
- persons with chronic illness predisposing to severe influenza such as:
 - cardiac disease, excluding simple hypertension
 - diabetes mellitus
 - chronic renal disease
 - haemoglobinopathies
 - immunosuppression, including that caused by cancers, medications or by HIV/AIDS infection
 - chronic neurological conditions

Staff surveillance

- A register of all staff entering the room of suspected or confirmed cases will be kept by the Nurse Manager (Refer to [Appendix 2.](#))
- The management of staff contacts exposed while unprotected will be discussed on a case by case basis with Occupational Safety and Health, the clinical microbiologist and a public health physician.
- Staff who have had contact with a suspected or confirmed cases should self-monitor for fever and other symptoms. Where relevant, post-exposure prophylaxis may be provided to staff members following individual assessment. This will be managed by Occupational Safety and Health in consultation with a clinical microbiologist.
- Exposed staff who develop symptoms of a respiratory illness must:
 - exclude themselves from the workplace
 - report the illness to their supervisor (who should inform the Infection Prevention and Control Unit and Occupational Safety and Health)
 - The period of exclusion from work will be determined in individual cases.

Related Documents

- [SCGOPHCG IPM Standard and transmission-based precautions](#)
- [SCGOPHCG IPM Influenza-Like Illness Policy](#)
- AS/NZS 1716:2012 Respiratory Protection Devices.

References

1. Communicable Disease Network Australia. Middle East Respiratory Syndrome Coronavirus (MERS-CoV). (2015). Retrieved from: <http://www.health.gov.au/internet/main/publishing.nsf/Content/cdna-song-mers-cov.htm>
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3. Australian Government. Australian Health Management Plan for Pandemic Influenza (AHMPPI). Department of Health. April 2014. [cited 2020 January30]. Available from: <http://www.health.gov.au/internet/main/publishing.nsf/Content/ohp-ahmppi.htm>
4. Centers for Disease Control and Prevention (CDC). Respiratory hygiene/cough etiquette in healthcare settings. 2009 [Updated 2012 Feb 27; [cited 2020 January30]. Available from:

<http://www.cdc.gov/flu/professionals/infectioncontrol/resphygiene.htm>

Authorisation

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Standards Applicable:					

For full details regarding, development, review, consultation, approval, endorsement - contact the Policy Officer for the submission form

APPENDIX 1: Collection of Specimens

Personal protective equipment (PPE) as per airborne precautions is to be worn when obtaining specimens.

EQUIPMENT REQUIRED:

- PPE: P2 (N95) mask – fit check must be performed, gloves, long sleeve fluid repellent gown and disposable full face visor
- Wooden tongue depressor
- Two flocked swab kits (Figure 1: each kit contains one flocked swab and one screw-top container of universal transport medium (UTM) – check expiry date before use.
- Blood tubes (SST plus other tubes for routine testing if required)



Figure 1: Flocked swab and UTM

SPECIMENS TO BE COLLECTED FOR TESTING

- One nasal swab (both nostrils) and one throat swab
- Lower respiratory tract (LRT) sample if the lower respiratory tract is involved (e.g. sputum, endotracheal aspirate, bronchoscopic sample – do not delay collecting and sending the nasal and throat swabs if a LRT specimen is not immediately available)
- SST blood tube for serology
- Send the samples to PathWest with an **urgent request for ‘Respiratory virus non-endemic (unless a specific name is available) PCR’** and specify the respiratory site and relevant clinical details, including travel details.
- **Do NOT use the pneumatic tube system.**

HOW TO COLLECT NASAL AND THROAT SWABS:

Hand hygiene is to be performed before and after specimen collection. PPE to be worn.

Nasal swab:

- Tilt the patient’s head back
- Gently insert the swab 2-3cm into the left nostril (the swab should be inserted towards the back of the head, not upwards - this is a swab of the nasal septum, not the anterior nares)
- Gently hold the swab against the nasal septum and rotate the swab twice
- Withdraw the swab and repeat process in the right nostril
- Place the swab into the UTM container, flocked end first, and snap the shaft at the indicated snapping point, using the rim of the UTM tube for leverage if required
- Close the UTM container with the flocked end of the swab inside, discarding the remainder of the shaft
- Ensure that the UTM container is closed correctly and is not leaking – the swab is usually slightly bent within the tube

Throat swab:

- Ask the patient to open their mouth and stick out their tongue. Use a wooden tongue depressor to press the tongue downwards
- Firmly swab the posterior pharynx and the tonsillar arches, rotating the swab twice while holding it against the tonsillar fossa at the side of the pharynx
- Withdraw the swab, which should be thoroughly wet with throat secretions
- Place swab into the UTM flocked end first, and snap the shaft at the indicated snapping point, using the rim of the UTM tube for leverage if required
- Close the UTM container with the flocked end of the swab inside, discarding the remainder of the shaft
- Ensure that the UTM container is closed correctly and is not leaking – the swab is usually slightly bent within the tube

Label the specimens and place both in a dual compartment biohazard bag and send to the laboratory. **Do NOT use the pneumatic tube system.**

APPENDIX 2: Staff Register**Contact with suspected or confirmed case of respiratory viruses (non-endemic)****Patient label****Staff Register****Date:**

	Staff member 1	Staff member 2	Staff member 3	Staff member 4
NAME				
Designation				
Start date				
Temperature/Signs and symptoms				
Day 1				
Day 2				
Day 3				
Day 4				
Day 5				
Day 6				
Day 7				
Day 8				
Day 9				
Day 10				
Day 11				
Day 12				
Day 13				
Day 14				